

Novo Nordisk A/S

NOVO-B.CO • NASDAQ Copenhagen • Drug Manufacturers - General • Denmark

COMPANY PROFILE

Novo Nordisk develops, manufactures and commercialises medicines for obesity, diabetes and selected rare diseases. Its marketed portfolio is led by semaglutide products, including Wegovy, Ozempic and Rybelsus, alongside insulins and haemophilia therapies. The company operates a global peptide and biologics manufacturing network and invests in next-generation metabolic and rare-disease pipelines.

RECOMMENDATION

BUY

12-month horizon

CURRENT PRICE

300.15 DKK

as of report date

TARGET PRICE

404.50 DKK

12-month fundamental

IMPLIED UPSIDE

+34.8%

vs. current price

MARKET CAP

DKK 1,333.4 bn

shares × price

ENTERPRISE VALUE

DKK 1,353.8 bn

mcap + EV adjustments

P/E 2026E

12.2×

EV/EBITDA 2026E

9.1×

FCF YIELD 2026E

9.8%

DIVIDEND YIELD
2026E

3.9%

NET DEBT / EBITDA
2026E

0.1×

RESEARCH SNAPSHOT

The call, the math, and the three reasons why

02

A one-page summary of the recommendation and the evidence behind it; the following pages provide the detail.

RECOMMENDATION BUY 12-month horizon	TARGET PRICE 404.50 DKK 12-month fundamental	CURRENT PRICE 300.15 DKK as of report date	IMPLIED UPSIDE +34.8% vs. current price
MARKET CAP DKK 1,333.4 bn shares × price	ENTERPRISE VALUE DKK 1,353.8 bn mcap + EV adjustments	P/E · EV/EBITDA 2026E 12.2x · 9.1x history + peers, see p. 18–19	FCF · DIV YIELD 2026E 9.8% · 3.9%



01 **Cash engine priced below its history**

The 2026 model values Novo at 9.14x EV/EBITDA versus a 21.61x normalised historical average. The report applies lower division multiples yet still derives material upside.

9.14x

02 **Access can offset price compression**

The Medicare bridge, French reimbursement and oral Wegovy expand the treated population. The valuation requires patient and persistence growth, not preservation of earlier list prices.

4.1m patients

03 **Pipeline value is not assumed at certainty**

Next-generation obesity, label expansion and rare-disease opportunities are valued through success cases, with substantial zero-value failure outcomes.

45–60% failure

**THESIS
BREAKER**

The investment case breaks if obesity share, net price and persistence deteriorate together quickly enough to push group EBIT margin below 35% before 2029.

SHARE PRICE DEVELOPMENT

Price trajectory and valuation anchors

03

Weekly closing prices over 36 months with the 12-month target price, alongside key market data.

Share price — last 36 months

DKK per share, with 12-month target line



CURRENT PRICE

300.15 DKK

20 August 2026

TARGET PRICE

404.50 DKK

12-month

IMPLIED UPSIDE

+34.8%

vs. current

52-WEEK HIGH

409.95 DKK

past 12m

52-WEEK LOW

224.25 DKK

past 12m

1-YEAR CHANGE

-13.2%

price only

3-YEAR CHANGE

-52.0%

price only

REPORT DATE

20 August 2026

READING THE MOVE

The rerating reflects slower 2025 growth, Lilly's U.S. prescription lead and CagriSema's failure to show non-inferiority to tirzepatide. Support later came from improved 2026 guidance, rapid oral Wegovy adoption and broader reimbursement. A complete attribution of every price move is unavailable. The DKK 408.6 target assumes neither a return to peak multiples nor a full competitive recovery.

VALUE BREAKDOWN · PART 1

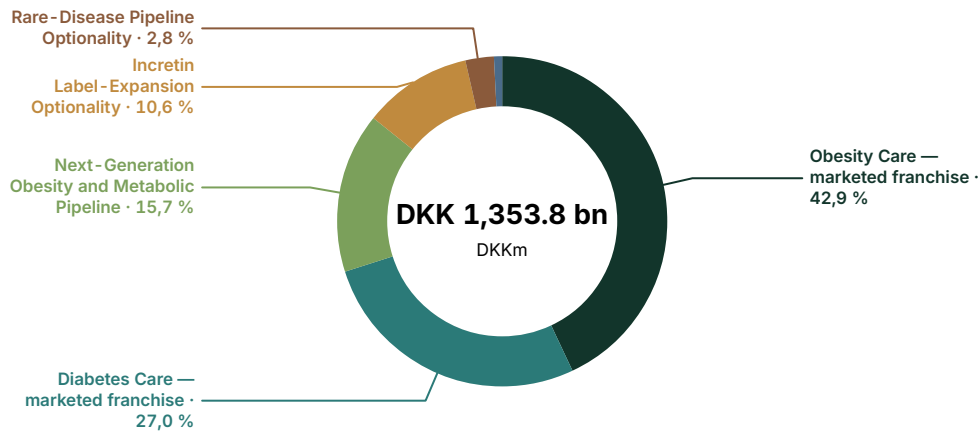
Enterprise - value allocation by business division

04

Where the market value sits across current businesses and long-term strategic options.

Enterprise value by business division

DKK millions · current analytical allocation



VALUE AND EARNINGS BY BUSINESS DIVISION

Analytical enterprise - value allocation against reported economics. "Reported" rows match a verified company - reported segment; "Modelled allocation" rows are model - side splits. Business divisions may differ from company - reported accounting segments.

	EV	EV %	REVENUE	REV %	REPORTED EBIT	REPORTED EBIT % OF TOTAL	BASIS
Obesity Care — marketed franchise	581,000	42.9%	130,255	42.1%	82,190	64.4%	Modelled allocation
Diabetes Care — marketed franchise	365,800	27.0%	159,201	51.5%	44,256	34.7%	Modelled allocation
Next-Generation Obesity and Metabolic Pipeline	212,300	15.7%	0	0.0%	0	0.0%	Modelled allocation
Incretin Label-Expansion Optionality	143,800	10.6%	0	0.0%	0	0.0%	Modelled allocation
Rare-Disease Pipeline Optionality	37,607	2.8%	0	0.0%	0	0.0%	Modelled allocation
Rare Disease — marketed franchise	13,300	1.0%	19,608	6.3%	1,212	0.9%	Reported
Total	1,353,807	100.0%	309,064	100.0%	127,658	100.0%	—

VALUE BREAKDOWN · PART 2

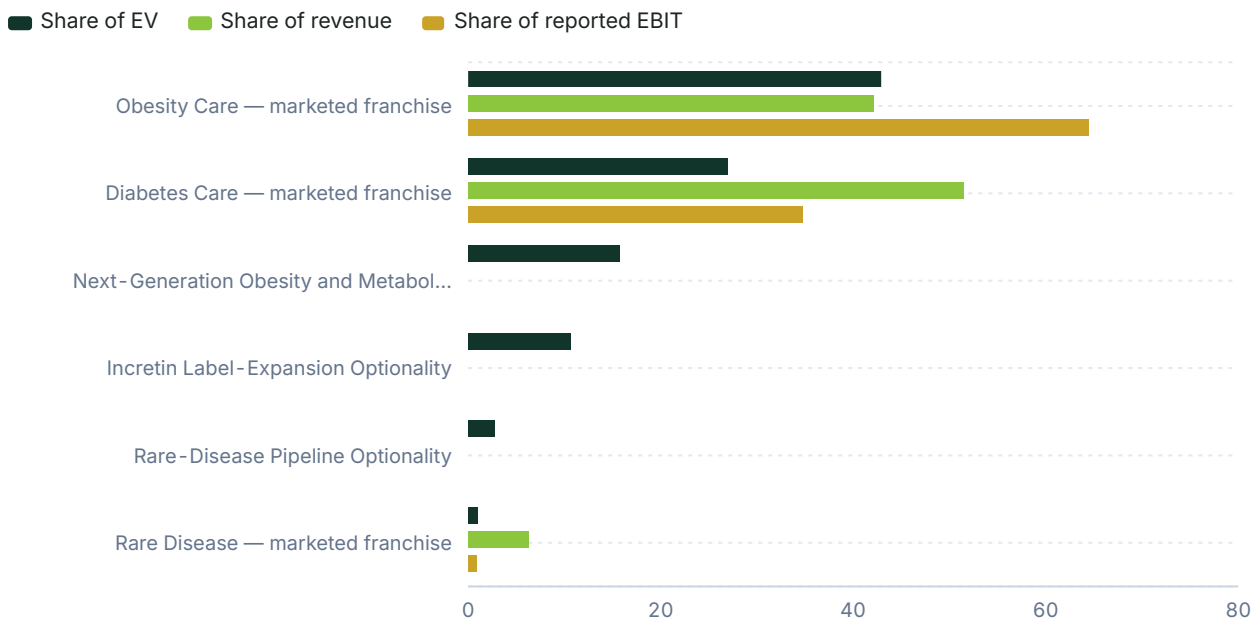
Where value sits versus where money is earned

05

Each business division's share of enterprise value against its share of group revenue and profit. A division holding value it does not yet earn is where the valuation rests on the future; the full figures are in the allocation table on the previous page.

Share of enterprise value vs revenue and profit

Per cent of group total · modelled allocation



INTERPRETATION

Current EV is allocated mainly to marketed obesity and diabetes, while approximately 29% is assigned to three probability-weighted opportunity pools. This is not a market-reported valuation. It is the report's economic decomposition of the fixed DKK 1,353.807 billion EV control total.

RECOMMENDATION

Why we land on the call we do

06

How the assembled evidence adds up to the call: what the price assumes, what the clues say, and what would change our view. The decision summary and the three headline reasons are on the snapshot page.

BUY. The 12-month target price is DKK 404.5, implying 34.8% upside from the current price of DKK 300.15.

At DKK 300.15, Novo's market capitalisation is DKK 1,333.4 billion and EV is DKK 1,353.8 billion. This price is justified if marketed franchises face sustained U.S. share and price erosion and most pipeline programmes fail to earn excess returns. The reverse DCF reaches the market price with 3% terminal growth, but assumes EBIT margin falls from 42.1% in 2026 to 24.5% terminally. Long-term margin durability is therefore the central debate.

The evidence is mixed. Novo held 39.9% of U.S. branded obesity prescriptions in the week ending 14 August 2026, versus Lilly's 60.1%, according to IQVIA data reported by Fierce Pharma. CagriSema also failed non-inferiority against tirzepatide. Offsetting this, obesity-care patients reached 4.1 million in Q1 2026, oral Wegovy captured about 90% of the U.S. oral segment, and management improved 2026 adjusted sales guidance to a decline of 0% to 6% at constant exchange rates.

The strongest case against BUY is that Novo may be moving from scarcity economics to a price-and-efficacy contest where it no longer leads. The 2027 U.S. list-price reduction to USD 675, the USD 274 Medicare negotiated price, Lilly's prescription lead and retatrutide's 28.7% Phase 3 weight loss could make the modelled DKK 75 billion obesity EBITDA unsustainable. Even so, the target values the marketed obesity business at 10.5x EBITDA, less than half Novo's normalised historical EV/EBITDA, and assigns a 55% failure outcome to next-generation obesity value.

A sustained fall below roughly one-third of U.S. branded obesity prescriptions, alongside group EBIT margin moving toward 35% before 2029, would shift the rating toward HOLD or SELL. Conversely, reimbursed patient growth, stable net revenue per persistent patient, clean oral manufacturing execution and pivotal amycretin superiority would support higher obesity multiples and pipeline probabilities.

CORE ANALYSIS · EXECUTIVE MAP

07

How the company creates economic value

The framework underlying the business division deep dives that follow.

EV is driven first by marketed obesity, where the key measure is persistent reimbursed patient-years, not prescriptions or list price. The report allocates DKK 581.0 billion of current EV to this business and values DKK 75 billion of modelled 2026 EBITDA at 10.5x in the target bridge. The franchise can absorb lower net prices if access, supply and persistence rise together.

Diabetes contributes DKK 365.8 billion of current EV. The target values DKK 62 billion of modelled 2026 EBITDA at 8.0x. Rare Disease is financially small and must restore its reported 6.2% EBIT margin. The three pipeline pools account for DKK 393.7 billion of current EV and are valued only through probability-weighted, discounted success cases.

The key group issue is whether EBIT margin remains near 40% through the competitive reset. The followed model forecasts 41.0% in 2026, 40.3% in 2027 and 40.8% in 2028, before 33.8% in 2029. The reverse valuation supports DKK 300 only by allowing terminal EBIT margin to decline to 24.5%. Durable margins therefore matter more than one year's revenue growth.

ANALYTICAL ANCHOR

The investment case is persistent patient-years × sustainable net revenue × contribution margin. Pipeline value is included only after probability weighting and time discounting.

08.1

Obesity Care — marketed franchise

Business division deep dive — Profit engine.

THE PRIZE IS OBESITY-RELATED HEALTHCARE SPENDING, NOT JUST TODAY'S ANTI-OBESITY DRUG MARKET

Obesity Care competes for healthcare spending currently absorbed by obesity and its complications. A U.S. managed-care study published through PubMed in July 2025 estimated attributable annual medical spending of USD 260.5 billion. Another market estimate put the figure at USD 173 billion. The report weights the former because it uses a defined medical-cost methodology, but uses the lower figure as a downside boundary. Labelled drug-market estimates are much smaller and inconsistent: IQVIA estimated USD 66 billion of 2025 list-price sales and USD 92 billion in 2026, whereas Fortune Business Insights estimated only USD 7.48 billion for 2025. The figures differ because list-price medicine sales, net pharmaceutical revenue and narrowly defined anti-obesity categories are not equivalent. The valuation therefore begins with displaced medical spending, but limits Novo's obtainable revenue to a share payers can justify through avoided complications.

The global economic pool is larger. The World Obesity Federation projected annual economic impact of USD 4.32 trillion, equal to 3% of global GDP. OECD evidence in the register identifies USD 425 billion of disease-treatment costs across 52 countries. Neither figure is treated as a drug-market forecast. They show that demand is not the constraint; reimbursement, persistence, tolerability, supply and sustainable net price are. The base valuation assumes Novo captures annual obesity revenue equal to less than 1% of the broad global economic burden and only a modest share of the directly addressable medicine market.

NOVO NEEDS MANY PATIENTS, BUT ONLY A CONSERVATIVE SHARE OF THE ECONOMIC POOL

The report models FY2025 obesity revenue at DKK 130.3 billion, or 45% of the DKK 289.5 billion reported Obesity and Diabetes care segment. This is an analyst estimate because Novo does not report obesity EBIT separately and the evidence package lacks a full product-sales schedule. The related DKK 82.2 billion EBIT allocation equals 65% of reported segment EBIT, reflecting the assumed mix advantage of branded obesity products over mature insulin. These allocations reconcile exactly to the reported segment and are not company-reported division figures.

OBESITY CARE — MARKETING FRANCHISE FACT SHEET

PROFIT ENGINE

MODEL ESTIMATE REVENUE
DKK 130,255m

MODEL ESTIMATE REPORTED EBIT
DKK 82,190m

MODELLED FY2025 EBIT MARGIN
63.1%; allocated within reported segment

VALUATION METRIC
DKK 75.0bn 2026E EBITDA × 10.5x

PATIENTS REACHED
4.1m in Q1 2026; +58% YoY

U.S. BRANDED SCRIPT SHARE
39.9% at 14 Aug 2026

U.S. ANNUAL NET-COST ANCHOR
USD 6,830 per patient

PERSISTENCE
62.7% at one year; 8.1% at three years

CAPACITY REQUIREMENT
Support >5m persistent patients without shortages

2027 PRICE RESET
USD 675 WAC; USD 274 Medicare

08.1

Obesity Care — marketed franchise (continued)

Business division deep dive — Profit engine.

ICER estimated annual U.S. net cost of USD 6,830 per Wegovy patient, or about USD 569 per month. Applied mechanically to 4.1 million patients, this implies USD 28.0 billion of annualised gross patient value. However, it overstates booked revenue because the patient figure is global, treatment duration and doses vary, and U.S. pricing cannot be applied worldwide. The practical conclusion is that sustaining DKK 130 billion of franchise revenue requires millions of persistent patients, not a small high-price niche. In the base case, prices decline but treated and persistent patient-years rise enough to keep obesity EBITDA around DKK 75 billion.

INJECTABLE SHARE HAS WEAKENED, BUT ORAL SEMAGLUTIDE OPENS A SECOND ACCESS CHANNEL

- Novo held 39.9% of U.S. branded obesity prescriptions in the week ending 14 August 2026, versus 60.1% for Lilly's Zepbound.
- Oral Wegovy held approximately 90% of the U.S. oral GLP-1 segment in August 2026, while Lilly's Foundayo generated USD 98 million in Q2 2026.
- Novo reached 4.1 million obesity-care patients in Q1 2026, up 58% from 2.6 million one year earlier.
- Novo's International Operations GLP-1 volume share was 58% in Q2 2026, compared with 59.6% global branded volume share at the end of 2025; these are different geographies and definitions, not directly comparable measures.

U.S. prescription data show Lilly leading branded obesity volume. Oral data show that the competitive outcome differs by formulation: Novo's first-mover position in tablets has opened a channel for needle-averse, self-pay and primary-care patients. This channel is less valuable per patient if the reported industry estimate of roughly 35% oral gross margin is directionally correct, versus more than 90% for injectable Wegovy. The estimate is not company disclosure and may omit scale benefits, but it reflects a real trade-off: oral semaglutide uses substantially more active ingredient and can expand access while reducing product margin.

The report does not interpret the 58% International Operations share and 59.6% 2025 global branded share as a precise 1.6-point decline. One is a regional GLP-1 volume measure and the other is a global branded-volume measure. Novo remains strong outside the U.S., while Lilly leads the most profitable and visible U.S. prescription market. This divergence supports a lower multiple than the franchise earned during supply-constrained hypergrowth.

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Obesity Care — marketed franchise (continued)

Business division deep dive — Profit engine.

ACCESS IS EXPANDING AS REALISED PRICES RESET LOWER

Headline 2027 U.S. economics are unusually transparent. Novo will reduce the list price of Wegovy and Ozempic to USD 675 per month from January 2027. Medicare's negotiated 30-day price is USD 274, a 71% discount to the referenced 2024 list price. The Medicare GLP-1 Bridge gives eligible beneficiaries access at a USD 50 monthly copay through the end of 2027. These figures are not interchangeable: WAC is not net manufacturer revenue, the negotiated price applies to a defined channel, and patient copay is not the amount retained by Novo.

European reimbursement is moving in the opposite direction to U.S. price compression. France introduced permanent 65% reimbursement in June 2026 for tightly selected patients after failed diet intervention, while the U.K. and EU authorised oral Wegovy during 2026. The valuation assumes broader eligibility and convenience increase patient-years faster than blended net revenue per patient declines. The operating equation is simple: revenue equals eligible starts multiplied by access, persistence and net revenue per persistent patient. If any two of these four variables disappoint at once, volume growth will not offset price erosion.

PERSISTENCE TURNS PRESCRIPTIONS INTO ENTERPRISE VALUE

Prime Therapeutics and Blue Cross Blue Shield data showed one-year Wegovy persistence improving to 62.7% in 2024 from 33.2% in 2021. However, only 8.1% of users remained on treatment after three years. The first figure supports near-term refill economics; the second challenges terminal value. A chronic medicine with high initiation and low long-term persistence can report rapid patient growth while replacing much of its revenue base each year. This raises selling expense, increases the importance of formulary access and reduces the value of each initial prescription.

The base case assumes one-year persistence remains above 55% and multi-year persistence improves as supply, dose flexibility and tolerability management improve. It does not assume three-year persistence approaches one-year persistence. At illustrative annual net revenue of USD 4,000–7,000 and five million persistent patient-years, gross annual patient value would be USD 20–35 billion. This is a report estimate, not a disclosed forecast. The range shows how a modest change in persistence can move revenue by tens of billions of kroner without changing addressable prevalence.

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Obesity Care — marketed franchise (continued)

Business division deep dive — Profit engine.

SUPPLY IS MOVING FROM A SCARCITY ADVANTAGE TO A RETURN-ON-CAPITAL TEST

Novo is investing more than DKK 60 billion in Kalundborg API capacity through 2029, DKK 15.9 billion in Hillerød, EUR 432 million in Athlone oral tableting, and USD 4.1 billion in a Clayton finished-product facility. Three acquired former Catalent fill-finish sites are expected to add capacity from 2026. These investments should reduce lost prescriptions and support tablets, higher doses and next-generation molecules. They also raise depreciation, execution risk and the capital base future cash flows must cover.

Manufacturing evidence is mixed. The Bloomington, Indiana facility received an Official Action Indicated classification after an April 2026 reinspection. The FDA also issued a warning letter concerning postmarketing adverse-event reporting at Plainsboro. Exact site utilisation and saleable dose capacity are undisclosed. The valuation assumes additional capacity becomes commercially productive rather than simply shifting bottlenecks from fill-finish to API, quality release or packaging.

COMPETITIVE EFFICACY LIMITS THE MULTIPLE EVEN IF THE MARKET GROWS

Lilly's retatrutide reported 28.7% average weight loss in the TRIUMPH-4 Phase 3 trial. CagriSema delivered 22.7% in REDEFINE 1 and approximately 23% in REDEFINE 4, but failed non-inferiority to tirzepatide. The result triggered a 15% share-price fall. Novo's high-dose Wegovy 7.2 mg achieved 20.7% mean weight loss and received EMA approval, providing a useful lifecycle extension but not clear category leadership. Efficacy is not the only driver of adoption, but it influences physician choice, persistence and payer willingness to grant preferred access.

The base valuation applies 10.5x to DKK 75 billion of modelled 2026 obesity EBITDA, producing DKK 787.5 billion of target EV before pipeline options. The multiple is below Novo's 21.61x normalised historical EV/EBITDA and Lilly's 43.7x 2026 multiple. It is above mature diversified-pharma levels because the addressable patient population, oral channel and outcomes evidence still support growth. A sustained U.S. share below one-third or EBITDA below DKK 60 billion would justify an 8x–9x multiple and remove more than DKK 250 billion from EV.

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08.1

Obesity Care — marketed franchise (continued)

Business division deep dive — Profit engine.

WHAT WOULD BREAK THE OBESITY VALUATION

The valuation fails if patient growth is mainly low-margin oral use or temporary self-pay use, if three-year persistence stays near 8%, or if net prices fall faster than manufacturing and selling costs. It also fails if Lilly's efficacy lead produces durable formulary exclusivity rather than simply higher share. Regulatory disruption at acquired fill-finish sites would worsen the outcome by preventing Novo from converting access gains into saleable doses. Conversely, stable net revenue per patient, five million-plus persistent patients and improved oral gross margin would make the 10.5x multiple conservative.

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OBESITY COMPETITION, EFFICACY AND ACCESS INDICATORS

COMPANY/PRODUCT	MARKET POSITION	EFFICACY OR SCALE	PRICE/ACCESS SIGNAL
Novo Wegovy franchise	39.9% U.S. scripts, Aug-26	4.1m patients, Q1-26	USD675 WAC from Jan-27
Lilly Zepbound	60.1% U.S. scripts, Aug-26	U.S. market leader	USD50 Medicare cap, Apr-26
Lilly retatrutide	Phase 3; pre-launch	28.7% weight loss	No commercial price yet
Lilly Foundayo	Oral challenger	USD98m sales, Q2-26	First full quarter
Bariatric surgery	177,297 U.S. cases, 2024	>20% decline YoY	Procedure alternative

08.2

Diabetes Care — marketed franchise

Business division deep dive — Profit engine.

DIABETES CARE COMPETES FOR A HEALTHCARE BURDEN FAR LARGER THAN THE DRUG MARKET

Diabetes Care addresses a global disease burden estimated at up to USD 152 trillion over 2020–2050, including informal care, or roughly 1.7% of annual global GDP. The commercial drug market is much smaller and definitions differ: Precedence Research estimated USD 75.09 billion for 2025, while Grand View Research estimated USD 104.0 billion and USD 117.5 billion for 2026. The report uses neither as a precise revenue denominator because one may define diabetes drugs narrowly and the other the wider antidiabetics market. The valuation instead considers whether Novo can preserve a high-value share of GLP-1 and insulin spending as statutory pricing, generics and therapeutic substitution increase.

Novo's modelled FY2025 Diabetes Care revenue is DKK 159.2 billion, equal to 55% of the reported DKK 289.5 billion Obesity and Diabetes care segment. Modelled EBIT is DKK 44.3 billion, or 35% of segment EBIT. The split is deliberately less profitable than obesity because it includes mature insulins, Victoza erosion and greater statutory-price exposure. It is an analyst allocation that reconciles to reported segment totals, not a disclosed division result.

THE FRANCHISE MUST DEFEND VALUE, NOT JUST UNITS

Novo reported a 30.1% global diabetes value share at the end of 2025, down 3.6 percentage points. A separate annual-report figure of 45.8% appears to cover a different segment or market definition; the report weights the explicitly described total global diabetes value share of 30.1%. Novo also reported 43% global GLP-1 volume share, while another filing cited 54.6% across diabetes and obesity care. These measures are not substitutes: the former covers the GLP-1 therapy market and the latter combines indications.

The share trend matters because value share can fall faster than prescription share as rebates, statutory discounts and mix shift toward lower-priced channels increase. Conversely, a company can lose percentage share while revenue grows if the category expands enough. The base case assumes Novo remains a major global diabetes supplier but no longer earns a scarcity or unchallenged-innovation premium. The division is therefore valued at 8.0x EBITDA, versus 10.5x for marketed obesity.

DIABETES CARE — MARKETED FRANCHISE FACT SHEET

PROFIT ENGINE

MODEL ESTIMATE REVENUE
DKK 159,201m

MODEL ESTIMATE REPORTED EBIT
DKK 44,256m

MODELLED FY2025 EBIT MARGIN
27.8%; allocated within reported segment

VALUATION METRIC
DKK 62.0bn 2026E EBITDA × 8.0x

GLOBAL DIABETES VALUE SHARE
30.1% at FY2025; -3.6ppt

GLOBAL GLP-1 VOLUME SHARE
43% in 2025

INSULIN SHARE ESTIMATE
45.3% globally in 2025

OZEMPIC LIST PRICE
About USD 935/month in 2026

AWIQLI STATUS
FDA approved 26 Mar 2026

PATENT PRESSURE
Semaglutide core U.S. patent Oct-26

08.2

Diabetes Care — marketed franchise (continued)

Business division deep dive — Profit engine.

TIRZEPATIDE HAS CHANGED THE COMPETITIVE BENCHMARK

- Lilly's Mounjaro generated USD 9.94 billion in Q2 2026, up 91% year on year, compared with reported Ozempic revenue of USD 4.88 billion for the same period.
- Lilly held roughly 60% of total U.S. GLP-1 prescriptions in Q2 2026 and approximately 70% of injectable GLP-1 prescriptions.
- Novo retained 43% of global GLP-1 volume in 2025 and 30.1% of total diabetes value.
- Novo is estimated to hold 45.3% of the global insulin market, while the top three insulin suppliers control about 90%.

Novo's diabetes business is not a single declining asset. Ozempic faces a stronger incretin competitor, but the insulin portfolio remains embedded in a concentrated global market and serves patients for whom GLP-1 therapy is unsuitable or insufficient. The portfolio also includes oral Rybelsus and the first once-weekly basal insulin approved in the U.S., providing lifecycle and convenience levers. The risk is that these products protect volume while lower prices and launch spending dilute incremental profit.

PRICE ARCHITECTURE IS MOVING TO LOWER VISIBLE PRICES WITH UNCERTAIN NET EFFECTS

Ozempic's cited U.S. list price is approximately USD 935 per month. Novo will reduce the WAC of Ozempic, Wegovy and Rybelsus to USD 675 from January 2027. Medicare's negotiated semaglutide price is USD 274 for a 30-day supply. An academic analysis estimated physical semaglutide manufacturing cost at USD 0.89–4.73 per month, but this excludes discovery, clinical trials, quality systems, device development, distribution, selling expense, rebates and failed programmes. It shows physical scalability, not an achievable net margin.

Exact net revenue per pen after PBM rebates and 340B adjustments is unavailable. The report assumes lower list prices are partly, but not fully, offset by reduced gross-to-net leakage. A 10% reduction in the division's modelled DKK 159.2 billion revenue, with a 60% decremental margin, would reduce EBIT by about DKK 9.6 billion. This is the report's sensitivity estimate. At 8.0x EBITDA, this would remove approximately DKK 77 billion of EV before any multiple contraction.

AWIQLI CAN SUPPORT INSULIN RELEVANCE BUT CANNOT REVERSE THE PRICING CYCLE

The FDA approved Awiqli, insulin icodec, on 26 March 2026 as the first once-weekly basal insulin for type 2 diabetes. The U.S. launch covers more than 70,000 pharmacies and offers eligible patients a payment as low as USD 35 per month. Weekly administration may improve convenience and persistence versus daily basal injections. It also gives Novo a differentiated replacement as Levemir is globally discontinued by the end of 2026.

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GLOBAL DIABETES VALUE SHARE
30.1% at FY2025; -3.6ppt

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43% in 2025

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Diabetes Care — marketed franchise (continued)

Business division deep dive — Profit engine.

The valuation assumes Awiqli slows insulin erosion rather than creating an obesity-like growth curve. Payer acceptance must reflect total treatment cost, hypoglycaemia management and adherence benefits, while the evidence package provides no early prescription data. Tresiba patent protection may extend to 2031 in the U.S., with biosimilar entry considered unlikely before 2032. This gives Novo time to migrate suitable patients, which is valuable because abrupt loss of exclusivity is usually more damaging than managed portfolio conversion.

PATENT AND GENERIC RISK IS GEOGRAPHICALLY UNEVEN

The core U.S. semaglutide composition-of-matter patent is estimated to expire in October 2026, while formulation and device patents may extend protection into 2031–2040. Novo reached a confidential settlement with Viatris and Natco over generic Ozempic, so the terms cannot be translated into a launch date. In India, the semaglutide patent expired in March 2026, enabling domestic manufacture. Victoza already faces an authorised U.S. generic launched by Teva in June 2024.

These facts argue against treating all semaglutide revenue as immediately generic, but also against assuming a perpetual premium. Device, formulation, manufacturing and litigation barriers can stagger erosion, but cannot remove payer leverage once credible alternatives emerge. The base case gives greater weight to price compression and share loss in markets with earlier entry, while assuming continued branded economics in protected channels. A clean global patent cliff would be materially worse than the model.

MANUFACTURING PROVIDES STRATEGIC CAPACITY BUT RAISES CAPITAL INTENSITY

Diabetes and obesity share API, tableting, device and fill-finish infrastructure. Novo expects approximately DKK 55 billion of 2026 capital expenditure to expand the global supply chain, following company-reported property, plant and equipment investment of DKK 60.1 billion in 2025. The model snapshot's DKK 2.45 billion gross capital expenditure for 2026 conflicts with management's operational guidance and is treated as a model-timing artefact. For valuation discipline, the report relies on EBITDA multiples and explicit debt rather than capitalising the unusually high model FCFF.

The shared network lowers unit costs and allows capacity to move across semaglutide indications, but it also creates cannibalisation and allocation decisions. A dose used for low-price oral obesity treatment cannot also serve a higher-value injectable diabetes patient while API remains constrained. Exact utilisation is undisclosed, so the model assumes capacity additions ease the bottleneck gradually through 2029. Delays would damage both Diabetes Care and Obesity Care, not one isolated business.

DIABETES CARE — MARKETED FRANCHISE FACT SHEET

PROFIT ENGINE

MODEL ESTIMATE REVENUE
DKK 159,201m

MODEL ESTIMATE REPORTED EBIT
DKK 44,256m

MODELLED FY2025 EBIT MARGIN
27.8%; allocated within reported segment

VALUATION METRIC
DKK 62.0bn 2026E EBITDA × 8.0x

GLOBAL DIABETES VALUE SHARE
30.1% at FY2025; -3.6ppt

GLOBAL GLP-1 VOLUME SHARE
43% in 2025

INSULIN SHARE ESTIMATE
45.3% globally in 2025

OZEMPIC LIST PRICE
About USD 935/month in 2026

AWIQLI STATUS
FDA approved 26 Mar 2026

PATENT PRESSURE
Semaglutide core U.S. patent Oct-26

08.2

Diabetes Care — marketed franchise (continued)

Business division deep dive — Profit engine.

VALUATION AND FAILURE CONDITIONS

The report values DKK 62 billion of modelled 2026 Diabetes Care EBITDA at 8.0x, producing DKK 496 billion of EV. The multiple is only modestly below Sanofi's 8.1x 2026 EV/EBITDA despite Novo's higher group margin, because diabetes faces concentrated semaglutide and U.S. price risk. It is below Amgen's 11.6x and AstraZeneca's 13.3x, and far below Lilly's 43.7x. This is a mature-franchise valuation with selective innovation credit, not a growth-stock valuation.

The division falls below the base valuation if global diabetes value share moves toward 25%, Mounjaro's lead widens outside the U.S., or semaglutide generic entry reaches protected markets earlier than expected. Awiqli's failure to secure preferred formulary access would remove an important insulin migration path. The upside case requires stable international GLP-1 volume, successful weekly-insulin conversion and net-price declines offset by lower rebates and mix. These conditions would support DKK 65–70 billion of EBITDA and a 9x–10x multiple.

DIABETES CARE — MARKETED FRANCHISE FACT SHEET

PROFIT ENGINE
MODEL ESTIMATE REVENUE DKK 159,201m
MODEL ESTIMATE REPORTED EBIT DKK 44,256m
MODELLED FY2025 EBIT MARGIN 27.8%; allocated within reported segment
VALUATION METRIC DKK 62.0bn 2026E EBITDA × 8.0x
GLOBAL DIABETES VALUE SHARE 30.1% at FY2025; -3.6ppt
GLOBAL GLP-1 VOLUME SHARE 43% in 2025
INSULIN SHARE ESTIMATE 45.3% globally in 2025
OZEMPIC LIST PRICE About USD 935/month in 2026
AWIQLI STATUS FDA approved 26 Mar 2026
PATENT PRESSURE Semaglutide core U.S. patent Oct-26

DIABETES FRANCHISE COMPETITIVE POSITION AND SUBSTITUTION RISK			
COMPANY/PRODUCT	MARKET POSITION	COMMERCIAL SCALE	DIFFERENTIATION/RISK
Novo diabetes portfolio	30.1% value share, 2025	DKK159.2bn modelled sales	GLP-1, oral and insulin breadth
Lilly Mounjaro	~60% U.S. GLP-1 scripts	USD9.94bn sales, Q2-26	Tirzepatide efficacy lead
Sanofi insulin	Share not in evidence	Diversified pharma base	Mature insulin benchmark
Jardiance alliance	55.3% SGLT2 estimate, 2026	Class-leading estimate	Cardiorenal substitution
AstraZeneca Farxiga	25% SGLT2 estimate, 2022	Global cardiorenal product	Non-incretin competitor

08.3

Next-Generation Obesity and Metabolic Pipeline

Business division deep dive — Emerging option.

THE PRIZE IS A FUTURE OBESITY MARKET ABOVE USD 100 BILLION ANNUALLY

Reuters reported in August 2026 that analysts expect obesity drugs to exceed USD 100 billion of annual sales by 2030, while IQVIA forecast USD 92 billion as early as 2026. Displaced medical and economic spending is much larger, but next-generation candidates must first compete in the treated medicine market. The report converts USD 100 billion into a DKK 750 billion 2030 addressable revenue pool using an explicit analytical currency assumption of DKK 7.5 per U.S. dollar. This exchange rate is not a reported forecast and is used only to make the option arithmetic auditable.

The moderate-success case assumes Novo captures 12% of this pool, or DKK 90 billion of revenue, and earns DKK 60 billion of EBITDA at scale in 2032. At DKK 30,000 of annual net revenue per persistent patient-year, this requires approximately three million patient-years. Both price and volume are report assumptions. The breakthrough case assumes 24% share, DKK 180 billion revenue and DKK 120 billion EBITDA. These outcomes sit below and above the centre of a plausible share range rather than presenting hidden point estimates.

CAGRISEMA APPEARS APPROVABLE BUT HAS NOT SHOWN CLEAR SUPERIORITY

CagriSema produced 22.7% mean weight loss at 68 weeks in REDEFINE 1, below Novo's prior 25% ambition. In REDEFINE 4, it produced approximately 23% at 84 weeks but failed non-inferiority against tirzepatide. Novo submitted the U.S. NDA in December 2025 and an FDA decision is anticipated in late 2026, although no confirmed PDUFA date is available. The cardiovascular outcomes trial REDEFINE 3 is expected to complete in October 2027.

Regulatory and commercial questions are distinct. Approval may be achievable based on absolute efficacy and safety, but the valuation requires superiority in efficacy, tolerability, persistence or convenience versus therapies available at launch. A product can be approved but fail to earn the premium economics assumed in a pipeline option. This is why the moderate-success probability is 30% and breakthrough probability only 15%, leaving 55% at zero.

AMYCRETIN OFFERS POTENTIAL DIFFERENTIATION BUT REMAINS LESS VALIDATED

Oral amycretin generated 13.1% mean weight loss after 12 weeks in Phase 1, an encouraging early signal for a combined GLP-1 and amylin mechanism. Novo planned a Phase 3 programme from 2026. The short duration, early stage and lack of comparative pivotal evidence prevent extrapolation to a commercial efficacy claim. Safety, dose escalation, manufacturing intensity and oral bioavailability remain material uncertainties.

NEXT-GENERATION OBESITY AND METABOLIC PIPELINE FACT SHEET

EMERGING OPTION

REVENUE

Not separately reported

REPORTED EBIT

Not separately reported

ADDRESSED 2030 MARKET

>USD 100bn annual obesity drugs

BASE SUCCESS SHARE ASSUMPTION

12% of modelled DKK 750bn market

BASE SCALE ECONOMICS

DKK 90bn revenue; 66.7% EBITDA margin

REQUIRED ANNUAL PATIENT-YEARS

About 3m at DKK 30,000 net revenue

CAGRISEMA EFFICACY

22.7–23.0% weight loss

AMYCRETIN EARLY EFFICACY

13.1% at 12 weeks

NEXT REGULATORY GATE

CagriSema FDA decision expected Q4 2026

FAILURE REMAINDER

55% in valuation

CORE ANALYSIS · BUSINESS DIVISION DEEP DIVES

Next-Generation Obesity and Metabolic Pipeline (continued)

08.3

Business division deep dive — Emerging option.

Amycretin may provide the unimolecular differentiation that CagriSema has not conclusively shown. Its failure risk is therefore high: early weight loss must persist in longer trials without unacceptable gastrointestinal or other adverse events. The option cases combine CagriSema, amycretin and related next-generation assets rather than assigning each programme a separate observable market value. Monlunabant receives no value after Novo recorded a DKK 4.0 billion impairment and earlier data showed neuropsychiatric side effects.

LILLY SETS A DEMANDING EFFICACY AND SCALE HURDLE

- Lilly's tirzepatide franchise generated more than USD 36 billion in 2025 and became the world's highest-selling drug franchise.
- Retatrutide achieved 28.7% average weight loss in Phase 3, above CagriSema's 22.7–23.0% results.
- Lilly held 60.1% of U.S. branded obesity prescriptions in August 2026.
- Amgen's MariTide, Viking's VK2735 and other pre-launch assets lack verified prospective market-share figures in the evidence.

The option is not valued on market growth alone. Novo must give physicians and payers a reason to select its next-generation product when tirzepatide, retatrutide, oral competitors and lifecycle-enhanced semaglutide are available. Better convenience could include monthly dosing, simpler escalation or an oral format, but exact patient preferences and manufacturing economics are unavailable. A similar injectable would likely cannibalise Wegovy rather than create the DKK 90–180 billion incremental revenue assumed in the success cases.

REGULATORY CASCADE AND NEXT GATES

CagriSema is pending in the U.S.; the evidence does not establish an EMA submission or approval. Amycretin's Phase 3 programme is the next clinical gate, but its EMA submission status is undisclosed. High-dose Wegovy and oral Wegovy are already approved in Europe and raise the commercial benchmark for a new molecule. The next events are the CagriSema FDA action, commercial label details, comparative tolerability and the start or readout of pivotal amycretin trials.

A narrow label or burdensome titration would reduce obtainable share even if approval occurs. A clean label with payer access could support the 12% moderate-success share. Demonstrated superiority approaching retatrutide efficacy with manageable tolerability would be needed for the 24% breakthrough share. The report does not assume approval alone unlocks either case.

NEXT-GENERATION OBESITY AND METABOLIC PIPELINE FACT SHEET

EMERGING OPTION

REVENUE

Not separately reported

REPORTED EBIT

Not separately reported

ADDRESSED 2030 MARKET

>USD 100bn annual obesity drugs

BASE SUCCESS SHARE ASSUMPTION

12% of modelled DKK 750bn market

BASE SCALE ECONOMICS

DKK 90bn revenue; 66.7% EBITDA margin

REQUIRED ANNUAL PATIENT-YEARS

About 3m at DKK 30,000 net revenue

CAGRISEMA EFFICACY

22.7–23.0% weight loss

AMYCRETIN EARLY EFFICACY

13.1% at 12 weeks

NEXT REGULATORY GATE

CagriSema FDA decision expected Q4 2026

FAILURE REMAINDER

55% in valuation

CORE ANALYSIS · BUSINESS DIVISION DEEP DIVES

Next-Generation Obesity and Metabolic Pipeline (continued)

08.3

Business division deep dive — Emerging option.

OPTION VALUATION

The moderate case values DKK 60 billion of 2032 EBITDA at 12.0x, producing DKK 720 billion of undiscounted EV with a 30% success probability. The breakthrough case values DKK 120 billion at 12.0x, producing DKK 1.44 trillion with a 15% probability. Discounting the probability-weighted amounts to 2026 at 7% gives approximately DKK 288 billion of present option value. The 55% failure outcome reflects the risk that pivotal evidence does not show commercially meaningful differentiation, even if one or more products are approved.

The current EV allocation of DKK 212.3 billion is below the report's target option value. This implies the market assumes lower probabilities, lower scale economics or greater cannibalisation. If the moderate case captures only 6% of the modelled market, its EBITDA and value halve. If amycretin shows clear superiority, the 15% breakthrough probability could rise materially before revenue appears. This is the highest-beta component of the target.

WHAT WOULD BREAK THE OPTION

The option approaches zero if CagriSema receives a restrictive outcome, amycretin fails in pivotal development, or successful products cannot earn attractive margins because high API use and price competition dominate. It is also impaired if next-generation sales mainly replace Wegovy and Ozempic rather than expand Novo's total share. The option rises if pivotal trials combine more than 25% weight loss, tolerable escalation, durable persistence and manufacturing efficiency. No single early efficacy datapoint is sufficient.

NEXT-GENERATION OBESITY AND METABOLIC PIPELINE FACT SHEET

EMERGING OPTION

REVENUE

Not separately reported

REPORTED EBIT

Not separately reported

ADDRESSED 2030 MARKET

>USD 100bn annual obesity drugs

BASE SUCCESS SHARE ASSUMPTION

12% of modelled DKK 750bn market

BASE SCALE ECONOMICS

DKK 90bn revenue; 66.7% EBITDA margin

REQUIRED ANNUAL PATIENT-YEARS

About 3m at DKK 30,000 net revenue

CAGRISEMA EFFICACY

22.7–23.0% weight loss

AMYCRETIN EARLY EFFICACY

13.1% at 12 weeks

NEXT REGULATORY GATE

CagriSema FDA decision expected Q4 2026

FAILURE REMAINDER

55% in valuation

NEXT-GENERATION OBESITY PIPELINE COMPETITIVE GATES

ASSET/COMPANY	CLINICAL SIGNAL	STATUS AT AUG-26	COMMERCIAL HURDLE
Novo CagriSema	22.7–23.0% weight loss	FDA review; decision expected	Failed tirzepatide non-inferiority
Novo amycretin	13.1% at 12 weeks	Phase 3 programme	Pivotal durability and safety
Lilly retatrutide	28.7% Phase 3 weight loss	Late-stage development	Efficacy benchmark
Amgen MariTide	Share unavailable	Clinical development	Long-acting differentiation
Viking VK2735	Share unavailable	Clinical development	Injectable/oral optionality

08.4

Incretin Label - Expansion Optionality

Business division deep dive — Emerging option.

THE PRIZE IS OUTCOME -BASED SPENDING IN KIDNEY, CARDIOVASCULAR AND LIVER DISEASE

J.P. Morgan and Forbes projected the global incretin market could reach approximately USD 200 billion annually by 2030 as adoption and label expansion broaden. This medicine-market figure sits against much larger disease-specific cost pools: U.S. Medicare kidney disease and kidney-failure spending reached USD 150 billion, cardiovascular costs were estimated at USD 100 billion by AHRQ or USD 223.2 billion by the CDC, and U.S. MASLD/MASH direct medical costs were estimated at USD 103 billion. The cardiovascular estimates use different scopes, so the report weights neither as a precise denominator. Together, they show that successful outcome labels can support reimbursement beyond cosmetic weight loss.

For valuation, the report converts USD 200 billion to DKK 1.5 trillion using an assumed DKK 7.5 exchange rate. The moderate case assumes 5% revenue capture, or DKK 75 billion, with DKK 45 billion EBITDA in 2032. The broad-success case assumes 10% capture, DKK 150 billion revenue and DKK 90 billion EBITDA. At DKK 30,000 annual net revenue per patient-year, these revenue levels require 2.5 million and five million patient-years respectively. These are transparent report assumptions.

EXISTING OUTCOME EVIDENCE SUPPORTS CKD AND MASH, BUT NOT EVERY PROPOSED INDICATION

The FDA expanded Ozempic's label in January 2025 to reduce kidney-disease progression, kidney failure and cardiovascular death in adults with type 2 diabetes and CKD. FLOW showed a 24% reduction in major kidney events and cardiovascular death. Wegovy received accelerated FDA approval for MASH with F2-F3 fibrosis in August 2025 and conditional U.K. approval in July 2026. These approvals provide commercial foundations rather than purely hypothetical science.

The evidence also includes failures. Oral semaglutide failed to improve cognition or function in early Alzheimer's disease in the Phase 3 EVOKE trials. Ziltivekimab's ZEUS cardiovascular trial failed its primary endpoint with a 0.99 hazard ratio. The report therefore does not extend semaglutide's metabolic benefits to universal disease modification. Each indication must earn probability and value through hard clinical outcomes, regulatory permission and reimbursed adoption.

INCRETIN LABEL - EXPANSION
OPTIONALITY FACT SHEET

EMERGING OPTION

REVENUE

Not separately reported

REPORTED EBIT

Not separately reported

2030 INCRETIN MARKET FORECAST

About USD 200bn annually

BASE ADDRESSED SPEND POOL

DKK 1,500bn at assumed 7.5 FX

MODERATE CAPTURE ASSUMPTION

5% revenue share; DKK 75bn

REQUIRED PATIENT-YEARS

2.5m at DKK 30,000 net revenue

CKD OUTCOME

24% event-risk reduction in FLOW

MASH STATUS

FDA 18 Aug 2025; U.K. 3 Jul 2026

FAILED PROGRAMMES

EVOKE and ZEUS unsuccessful

FAILURE REMAINDER

50% in valuation

08.4

Incretin Label-Expansion Optionality (continued)

Business division deep dive — Emerging option.

REIMBURSEMENT IS THE COMMERCIAL GATE AFTER APPROVAL

- The Medicare GLP-1 Bridge includes eligible beneficiaries with obesity and specified comorbidities, including HFpEF and CKD Stage 3a+, at a USD 50 monthly copay from July 2026.
- NICE-related evidence indicates 1.2 million people could be eligible for Wegovy in cardiovascular disease management in England.
- France permanently reimburses Wegovy at 65% for tightly defined severe-obesity patients from June 2026.
- Exact country-level CKD reimbursement across Europe remains unavailable.

These pathways show how label expansion can increase patient access while lowering net price. A cardiovascular or kidney label can move treatment into disease budgets with clearer avoided-cost evidence, but payers can use the larger eligible population to demand lower unit prices. The base case assumes scale grows faster than unit economics compress, producing a 60% EBITDA margin at maturity. This is below a scarcity-priced injectable gross margin and includes commercial and manufacturing costs.

COMPETITIVE SUBSTITUTION DIFFERS BY INDICATION

In MASH, Madrigal's Rezdiffra is the main incumbent and was associated with approximately USD 1 billion of annual revenue in 2025. Semaglutide may benefit from obesity, diabetes and cardiovascular familiarity, but MASH reimbursement depends on liver-specific outcomes, not weight loss alone. In CKD and cardiovascular disease, SGLT2 inhibitors such as Jardiance and Farxiga already occupy outcome-based treatment pathways. Novo must show additive or superior outcomes, not simply metabolic efficacy.

There are no established product market shares for incretin treatment of CKD, OSA or other emerging indication segments. The option therefore uses share of the broad 2030 incretin revenue pool, not invented indication-level shares. Cannibalisation is material: some CKD and cardiovascular patients already receive Ozempic or Wegovy under diabetes or obesity labels. The valuation cases represent incremental access and duration after an analytical 30% cannibalisation allowance within the EBITDA assumptions.

REGULATORY CASCADE

Approved evidence includes U.S. Ozempic for diabetic CKD, U.S. Wegovy for MASH, U.K. Wegovy for MASH, cardiovascular risk reduction in weight management, and selected European lifecycle extensions. Pending value depends on broader reimbursement, confirmatory MASH outcomes and disease-specific uptake. Alzheimer's receives zero option value after EVOKE. ZEUS receives zero incremental value after its failed primary endpoint.

INCRETIN LABEL-EXPANSION OPTIONALITY FACT SHEET

EMERGING OPTION

REVENUE

Not separately reported

REPORTED EBIT

Not separately reported

2030 INCRETIN MARKET FORECAST

About USD 200bn annually

BASE ADDRESSED SPEND POOL

DKK 1,500bn at assumed 7.5 FX

MODERATE CAPTURE ASSUMPTION

5% revenue share; DKK 75bn

REQUIRED PATIENT-YEARS

2.5m at DKK 30,000 net revenue

CKD OUTCOME

24% event-risk reduction in FLOW

MASH STATUS

FDA 18 Aug 2025; U.K. 3 Jul 2026

FAILED PROGRAMMES

EVOKE and ZEUS unsuccessful

FAILURE REMAINDER

50% in valuation

08.4

Incretin Label - Expansion Optionality (continued)

Business division deep dive — Emerging option.

The key next gate is not another prevalence estimate. It is observable prescription and persistence growth among outcome-qualified patients at a net price that provides an attractive contribution margin. Confirmatory MASH evidence also matters because accelerated approval rests on histological surrogate endpoints. Failure to confirm clinical benefit could restrict or reverse the opportunity.

OPTION VALUATION AND FAILURE CASE

The moderate case applies 10.0x to DKK 45 billion of 2032 EBITDA, yielding DKK 450 billion of undiscounted EV at 35% probability. The broad-success case applies 10.0x to DKK 90 billion, yielding DKK 900 billion at 15% probability. Discounted at 7% to 2026, the probability-weighted present value is approximately DKK 195 billion. The remaining 50% has zero value because approvals may not generate incremental reimbursed patient-years beyond the existing diabetes and obesity franchises.

The current EV allocation of DKK 143.8 billion implies greater scepticism than the report's target bridge. This is defensible given EVOKE, ZEUS, price negotiation and cannibalisation. The report differs because CKD and MASH already have positive outcomes or approvals, making complete economic failure less likely than for an unvalidated discovery programme. If incremental net revenue per patient falls below DKK 20,000 or cannibalisation exceeds half of labelled uptake, option value would decline sharply.

INCRETIN LABEL - EXPANSION
OPTIONALITY FACT SHEET

EMERGING OPTION

REVENUE

Not separately reported

REPORTED EBIT

Not separately reported

2030 INCRETIN MARKET FORECAST

About USD 200bn annually

BASE ADDRESSED SPEND POOL

DKK 1,500bn at assumed 7.5 FX

MODERATE CAPTURE ASSUMPTION

5% revenue share; DKK 75bn

REQUIRED PATIENT-YEARS

2.5m at DKK 30,000 net revenue

CKD OUTCOME

24% event-risk reduction in FLOW

MASH STATUS

FDA 18 Aug 2025; U.K. 3 Jul 2026

FAILED PROGRAMMES

EVOKE and ZEUS unsuccessful

FAILURE REMAINDER

50% in valuation

08.4

Incretin Label-Expansion Optionality (continued)

Business division deep dive — Emerging option.

WHAT BREAKS THE OPTION

The option fails if hard outcomes fail in confirmatory studies, payers value new labels no more highly than obesity treatment, or incremental use merely relabels existing semaglutide patients. It also fails if lower negotiated prices undermine the assumed 60% EBITDA margin. It strengthens if disease-specific reimbursement expands across major European systems and real-world persistence improves because patients view treatment as prevention of kidney, liver or cardiovascular events rather than weight management alone.

INCRETIN LABEL - EXPANSION OPTIONALITY FACT SHEET

EMERGING OPTION

REVENUE

Not separately reported

REPORTED EBIT

Not separately reported

2030 INCRETIN MARKET FORECAST

About USD 200bn annually

BASE ADDRESSED SPEND POOL

DKK 1,500bn at assumed 7.5 FX

MODERATE CAPTURE ASSUMPTION

5% revenue share; DKK 75bn

REQUIRED PATIENT - YEARS

2.5m at DKK 30,000 net revenue

CKD OUTCOME

24% event-risk reduction in FLOW

MASH STATUS

FDA 18 Aug 2025; U.K. 3 Jul 2026

FAILED PROGRAMMES

EVOKE and ZEUS unsuccessful

FAILURE REMAINDER

50% in valuation

LABEL - EXPANSION EVIDENCE, INCUMBENTS AND SPEND POOLS

INDICATION/COMPETITOR	OUTCOME OR POSITION	STATUS	ECONOMIC POOL
Ozempic in diabetic CKD	24% risk reduction	FDA approved Jan-25	USD150bn Medicare kidney spend
Wegovy in MASH	Histology surrogate benefit	FDA/U.K. approved	USD103bn U.S. direct costs
Rezdiffra in MASH	Primary incumbent	~USD1bn annual revenue	Liver-specific competitor
Jardiance	55.3% SGLT2 estimate	Established outcomes therapy	Cardiorenal substitution
Semaglutide in Alzheimer's	No cognition/function benefit	Phase 3 failed Mar-26	Zero value assigned

08.5

Rare-Disease Pipeline Optionality

Business division deep dive — Emerging option.

THE PRIZE COMBINES HAEMOPHILIA, SICKLE CELL DISEASE AND PNH SPENDING

The global haemophilia drug market was estimated at USD 10.2 billion in 2025, while the PNH market is forecast to reach USD 11.7 billion by 2034. The broader rare-disease treatment market exceeds USD 240 billion, but using this figure would overstate the opportunity because Novo's assets target defined blood disorders. Existing treatment costs can be very high, including ten-year haemophilia prophylaxis costs measured in seven figures. The option is therefore volume-limited but can be valuable per successfully treated patient.

The moderate case assumes DKK 16 billion of revenue and DKK 8 billion EBIT in 2031. At DKK 2 million annual net revenue per patient, this requires approximately 8,000 treated patient-years. Price and volume are report assumptions informed by the high cost of existing specialist therapies, not disclosed forecasts. The high-success case doubles revenue and EBIT. The analysis values combined portfolio economics because peak-sales estimates for Mim8 and zaltenibart were not available.

CLINICAL SIGNALS ARE STRONG, BUT REGULATORY OUTCOMES REMAIN INCOMPLETE

Mim8 demonstrated a 99% reduction in treated bleeding episodes with once-monthly dosing in haemophilia A patients without inhibitors. Regulatory applications were submitted in late 2025, but the evidence package does not establish whether the FDA approved or rejected the product by 20 August 2026. Etavopivat met both Phase 3 co-primary endpoints, including a 27% reduction in annualised vaso-occlusive crises, and Novo plans a filing in the second half of 2026. Zaltenibart is in Phase 3 PNH development with a BLA expected in Q4 2026.

These programmes are more advanced than the early obesity pipeline, supporting a combined 60% probability across success cases. The remaining 40% reflects product-specific regulatory, safety, launch and competitive risk, not a view that every programme fails together. Approval alone is insufficient: Mim8 must displace established haemophilia prophylaxis, etavopivat must show meaningful patient benefit beyond trial endpoints, and zaltenibart must compete in a specialist complement-inhibitor market.

COMMERCIAL FIT CAN SUPPORT MARGINS IF LAUNCHES USE EXISTING INFRASTRUCTURE

Novo already sells haemophilia and endocrine therapies through a specialist rare-disease organisation. Successful products can use this commercial base and potentially lift the division's low current margin. Once-monthly Mim8 offers a clear administration benefit, while etavopivat and zaltenibart broaden the addressable disease base. However, each launch requires education, payer evidence, diagnostics and postmarketing surveillance, so operating leverage may emerge slowly.

RARE-DISEASE PIPELINE
OPTIONALITY FACT SHEET

EMERGING OPTION

REVENUE

Not separately reported

REPORTED EBIT

Not separately reported

COMMERCIAL REFERENCE MARKET

USD 10.2bn haemophilia drugs in 2025

MODERATE SCALE ASSUMPTION

DKK 16bn revenue; DKK 8bn EBIT

REQUIRED TREATED POPULATION

8,000 at DKK 2m annual net revenue

MIM8 PIVOTAL SIGNAL

99% reduction in treated bleeds

ETAVOPIVAT PIVOTAL SIGNAL

27% lower VOC rate

ZALTENIBART MARKET

USD 11.7bn PNH forecast for 2034

NEXT FILINGS

Etavopivat H2-26; zaltenibart Q4-26

FAILURE REMAINDER

40% in valuation

08.5

Rare-Disease Pipeline Optionality (continued)

Business division deep dive — Emerging option.

The moderate valuation applies 12.0x to DKK 8 billion of 2031 EBIT at 45% probability. The high-success case applies 12.0x to DKK 16 billion at 15%. Discounted at 7%, the combined present value is approximately DKK 51 billion. The current EV allocation is DKK 37.6 billion, leaving a smaller valuation disagreement than in metabolic pipeline value.

REGULATORY CASCADE AND FAILURE CONDITIONS

Approved marketed comparators include Alhemo and Rivfloza, but their value sits in the marketed Rare Disease division. Mim8 remains pending because the evidence lacks a final FDA outcome. Etavopivat and zaltenibart await filing and review. The next gates are a verified Mim8 regulatory outcome, etavopivat submission and detailed zaltenibart Phase 3 evidence.

The option fails if Mim8's regulatory status proves adverse, if safety or monitoring prevents broad use, or if pivotal benefits do not translate into preferred reimbursement. It rises if Mim8 secures a broad label, etavopivat files on schedule and the three assets share commercial infrastructure without proportionate cost growth. As the pool is only 2.8% of current EV, it is not needed for the BUY case but provides asymmetric support.

RARE-DISEASE PIPELINE
OPTIONALITY FACT SHEET

EMERGING OPTION

REVENUE

Not separately reported

REPORTED EBIT

Not separately reported

COMMERCIAL REFERENCE MARKET

USD 10.2bn haemophilia drugs in 2025

MODERATE SCALE ASSUMPTION

DKK 16bn revenue; DKK 8bn EBIT

REQUIRED TREATED POPULATION

8,000 at DKK 2m annual net revenue

MIM8 PIVOTAL SIGNAL

99% reduction in treated bleeds

ETAVOPIVAT PIVOTAL SIGNAL

27% lower VOC rate

ZALTENIBART MARKET

USD 11.7bn PNH forecast for 2034

NEXT FILINGS

Etavopivat H2-26; zaltenibart Q4-26

FAILURE REMAINDER

40% in valuation

08.6

Rare Disease — marketed franchise

Business division deep dive — Profit engine.

THE PRIZE IS THE AVOIDABLE BURDEN OF UNTREATED RARE DISEASE

The broad rare-disease treatment market was estimated at USD 247.36 billion in 2025, while another source estimated USD 265.5 billion for 2026. The U.S. economic burden, including mortality and indirect costs, has been estimated at USD 7.2–8.6 trillion annually, but this is not a pharmaceutical revenue pool. Novo's marketed franchise competes more directly in haemophilia and endocrine disorders, including a USD 10.2 billion global haemophilia drug market. The valuation uses narrower commercial categories because Novo lacks a platform across the full rare-disease universe.

FY2025 reported Rare disease revenue was DKK 19.608 billion and reported EBIT was DKK 1.212 billion, equal to a 6.2% margin. These verified figures differ from snippets citing DKK 18.6 billion sales and a 3.3% margin. The report uses the independently verified segment table because it reconciles exactly to group revenue and EBIT. H1 2026 sales declined 2% at constant exchange rates, mainly due to lower NovoSeven volumes in China and EUCAN. The division must restore margins as well as grow revenue.

NEW ENDOCRINE PRODUCTS MUST OUTGROW ESTABLISHED HAEMOPHILIA EROSION

Rare endocrine sales, led by Sogroya, reached DKK 5.96 billion in 2025 and grew 24% at constant exchange rates. Novo reported 59.6% global human-growth-hormone volume share, while an external estimate put Norditropin at 34% of 2024 brand revenue. The measures only appear to conflict because one covers company volume share and the other revenue share for a specific brand. The report weights company-reported volume for market position but uses the lower revenue-share estimate to show that mix and pricing differ materially.

In haemophilia, Roche's Hemlibra generated more than USD 4.9 billion globally in 2024 and about USD 3 billion in the U.S. It is the benchmark against which Novo's factor and non-factor products compete. Takeda is estimated to lead the broad haemophilia treatment market with 15%, while the top five suppliers hold about 60%. Novo can grow without becoming the overall leader, but needs differentiated administration or efficacy rather than relying on legacy factor replacement.

PERMISSION AND MONITORING REMAIN PRODUCT-SPECIFIC

- Alhemo was approved by the FDA on 31 July 2025 for haemophilia A or B without inhibitors and by the EMA in September 2025; the EMA requires additional safety monitoring.
- Rivfloza has been marketed for primary hyperoxaluria type 1 since its September 2023 FDA approval.
- Sogroya was submitted in China for three additional indications in November 2025.
- Mim8 remained under regulatory review in early August 2026; the evidence package does not establish a final FDA decision.

RARE DISEASE — MARKETED FRANCHISE FACT SHEET

PROFIT ENGINE

FY2025A REVENUE
DKK 19,608m

FY2025A REPORTED EBIT
DKK 1,212m

REPORTED FY2025 EBIT MARGIN
6.2%

VALUATION METRIC
DKK 1.5bn 2026E EBIT × 12.0x

H1 2026 SALES TREND
-2% at CER

RARE ENDOCRINE SALES
DKK 5.96bn; +24% CER in 2025

GROWTH-HORMONE VOLUME SHARE
59.6% in 2025

HAEMOPHILIA LEADER
Hemlibra >USD4.9bn in 2024

ALHEMO STATUS
FDA and EMA approved in 2025

REQUIRED RECOVERY
EBIT margin above 10% by 2027

08.6

Rare Disease — marketed franchise (continued)

Business division deep dive — Profit engine.

The marketed division receives credit only for products already approved and generating, or capable of generating, near-term revenue. Mim8, etavopivat and zaltenibart remain in Rare-Disease Pipeline Optionality to avoid double counting. Alhemo's additional-monitoring status makes real-world safety and physician confidence important. The report assumes a measured launch rather than immediate displacement of Hemlibra.

ECONOMICS CAN IMPROVE SHARPLY FROM A LOW REPORTED BASE

Rare-disease medicines can command high prices because populations are small and untreated complications are costly. One cost simulation estimated nearly EUR 991,000 of annual NovoSeven cost for a 70-kilogram patient, while a haemophilia analysis estimated USD 1.7 million of ten-year factor VIII prophylaxis spending. These are not Novo net-revenue disclosures and should not be multiplied by prevalence. They show that payers assess rare-disease therapies against high existing treatment and complication costs.

Exact product COGS is unavailable. The segment's 6.2% EBIT margin shows that high prices do not automatically produce high current profit after manufacturing, commercial infrastructure, portfolio erosion and pipeline support. The valuation assumes reported EBIT rises to DKK 1.5 billion in 2026 and margin moves above 10% by 2027. Without this recovery, even the modest DKK 18 billion target EV would be hard to defend.

VALUATION AND BREAK CONDITIONS

The report applies 12.0x to DKK 1.5 billion of 2026 EBIT, producing DKK 18 billion of target EV. The multiple is below Sobi's 16.6x 2026 EV/EBIT but above Sanofi's 10.0x because Novo has differentiated launches and potentially stronger growth from a depressed margin. The division is too small to carry the group valuation. It can still provide upside if Sogroya, Alhemo and Rivfloza absorb the fixed commercial base.

RARE DISEASE — MARKETING FRANCHISE FACT SHEET

PROFIT ENGINE

FY2025A REVENUE
DKK 19,608m

FY2025A REPORTED EBIT
DKK 1,212m

REPORTED FY2025 EBIT MARGIN
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VALUATION METRIC
DKK 1.5bn 2026E EBIT × 12.0x

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HAEMOPHILIA LEADER
Hemlibra >USD4.9bn in 2024

ALHEMO STATUS
FDA and EMA approved in 2025

REQUIRED RECOVERY
EBIT margin above 10% by 2027

08.6

Rare Disease — marketed franchise (continued)

Business division deep dive — Profit engine.

The valuation fails if established NovoSeven and factor products erode faster than new products scale, or if Alhemo safety monitoring limits adoption. It improves if rare endocrine growth remains above 15%, Rare disease EBIT margin reaches the low teens and pipeline launches use existing commercial infrastructure. The abandoned DKK 8.5 billion Odense factory plan reduces near-term capital commitments but signals that management's earlier demand expectations changed.

RARE DISEASE — MARKETING FRANCHISE FACT SHEET

PROFIT ENGINE

FY2025A REVENUE
DKK 19,608m

FY2025A REPORTED EBIT
DKK 1,212m

REPORTED FY2025 EBIT MARGIN
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DKK 1.5bn 2026E EBIT × 12.0x

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Hemlibra >USD4.9bn in 2024

ALHEMO STATUS
FDA and EMA approved in 2025

REQUIRED RECOVERY
EBIT margin above 10% by 2027

RARE - DISEASE FRANCHISE BENCHMARKS			
COMPANY/PRODUCT	MARKET POSITION	SCALE	KEY DISTINCTION
Novo Rare Disease	6.3% group revenue, 2025	DKK19.608bn sales	6.2% reported EBIT margin
Roche Hemlibra	Haemophilia A leader	>USD4.9bn sales, 2024	Established non-factor therapy
Takeda haemophilia	15% market estimate, 2025	USD1-2bn factor VIII brands	Broad factor portfolio
Sobi	Top-five supplier	30.4% EBIT margin, 2026E	Pure rare-disease benchmark
Ascendis Skytrofa	6.5% U.S. GH share, 2024	Long-acting growth hormone	Sogroya competitor

CORE ANALYSIS · CROSS-DIVISION BRIDGE

Putting the divisions back together

09

How the business division valuations aggregate to the group, and the re-rating the target price requires.

Obesity and diabetes share semaglutide API, devices, fill-finish capacity and commercial access. Added capacity can serve both businesses, but constrained doses require a trade-off between higher-priced injections and lower-priced oral expansion.

The marketed franchises fund the pipeline and manufacturing programme. Pipeline successes can use the commercial network, but CagriSema and label expansions may cannibalise Wegovy or Ozempic. Gross product sales therefore cannot be treated as fully incremental.

Rare Disease has less manufacturing overlap but can use a common specialist organisation. Its pipeline matters mainly through margin leverage: new launches can absorb fixed infrastructure that currently supports only DKK 1.2 billion of reported EBIT.

BRIDGE MECHANICS

Profit engines are valued on modelled 2026 EBITDA or EBIT. Pre-revenue businesses are valued on future scale economics, multiplied by a sector-relevant EV multiple, weighted by success probability and discounted at 7%.

Novo trades at 9.14x 2026 EV/EBITDA and 12.21x P/E, well below its 21.61x and 29.39x normalised historical averages. The decline is partly justified: sales are forecast to fall 3.3% in 2026 and competition has intensified.

The broad 2026 peer median is 12.8x EV/EBITDA. Lilly's 43.7x is not an appropriate group benchmark, while Sanofi's 8.1x understates Novo's obesity and pipeline exposure. The SOTP avoids applying either extreme to every division.

The reverse DCF matches the current market capitalisation with 3% long-run growth and a 24.5% terminal EBIT margin. This is coherent if pricing and competition structurally halve the economics of the current franchise. It is demanding if manufacturing scale and outcome labels preserve high returns.

The target of DKK 408.6 per share is close to the DKK 415.4 ROE/PBV scatter indication. Agreement between the methods is supportive, not proof, because both depend on forecast returns remaining exceptional.

CORE ANALYSIS · SCENARIOS & VERDICT

10

Scenario logic and the bottom-line judgment

How the conclusion changes under less favourable scenarios.

Bear assumes DKK 270 billion of revenue, DKK 108 billion of EBITDA and a 40.0% margin. It assigns zero value to the pipeline, applies mature-pharma multiples to marketed assets and reflects simultaneous U.S. share loss, price compression and weak persistence.

Base uses followed 2026 revenue and EBITDA of DKK 298.825 billion and DKK 148.102 billion. It combines conservative marketed-franchise multiples with probability-weighted opportunities, producing an implied group multiple of about 12.4x.

Bull assumes DKK 360 billion of revenue and DKK 190 billion of EBITDA at a 52.8% margin. It values the strongest pipeline cases, faster oral and reimbursement adoption, improved capacity use and positive net cash generation, rather than holding base-case debt unchanged.

The scenarios are not simple multiple changes. Revenue, margin, pipeline success and balance-sheet outcomes move together because successful launches generate additional cash, while failure leaves capacity and commercial costs underabsorbed.

BOTTOM LINE

Novo no longer merits an uncontested hypergrowth multiple, but the current price appears to assume a deeper and more lasting margin decline than the evidence requires. The BUY case rests first on resilient marketed cash flow and second on discounted pipeline value. It does not require Novo to regain immediate U.S. prescription leadership.

FINANCIAL BRIDGE · PART 1

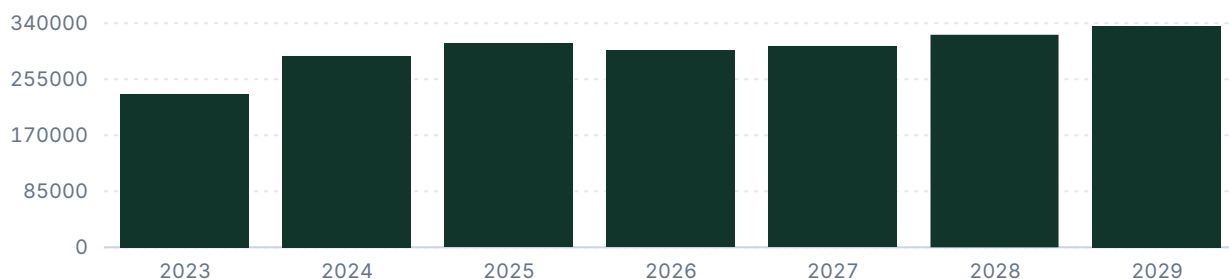
Net sales, EBITDA / EBIT, EBIT margin

14

Group-level trajectory for sales, earnings and margin; shaded bands mark forecast periods.

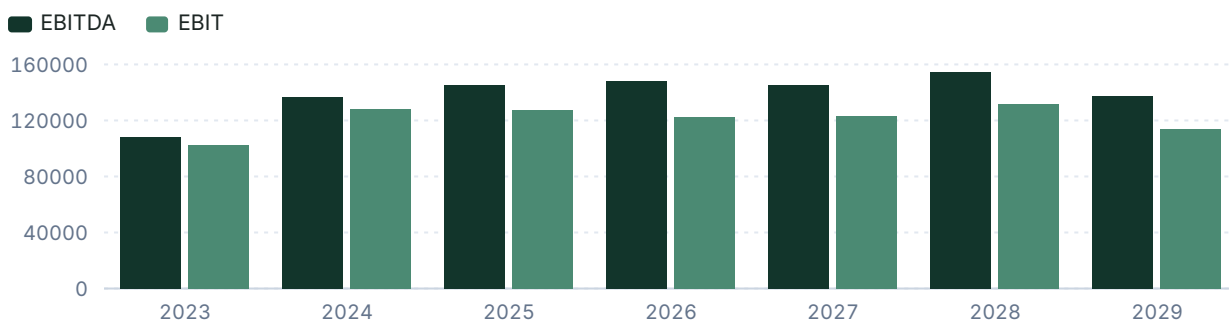
Net sales

DKK millions



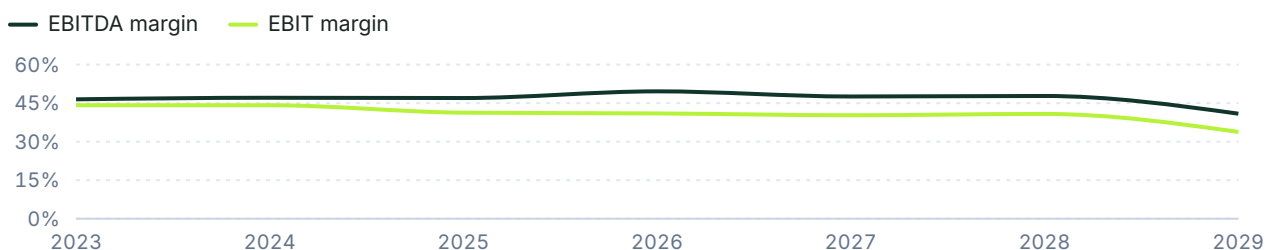
EBITDA & EBIT

DKK millions



EBIT and EBITDA margin

% of net sales



FINANCIAL BRIDGE · PART 2

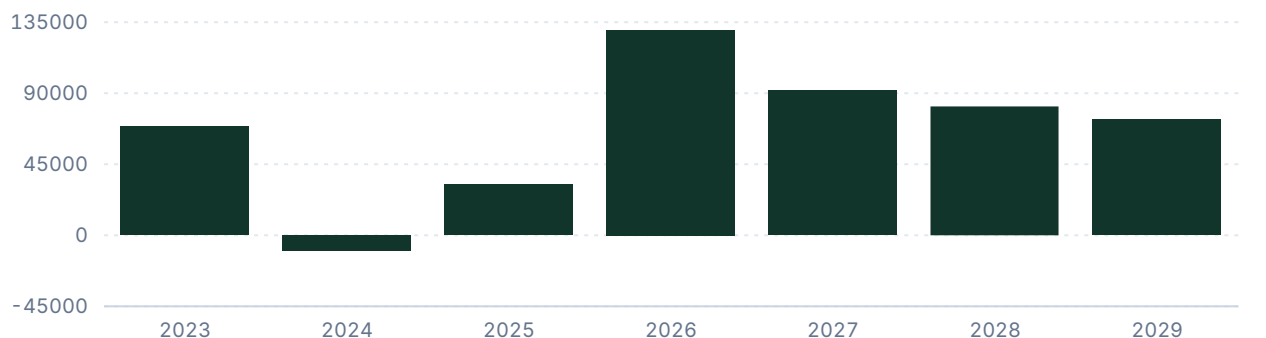
15

Free cash flow, leverage, and key financials

Cash generation and balance-sheet capacity.

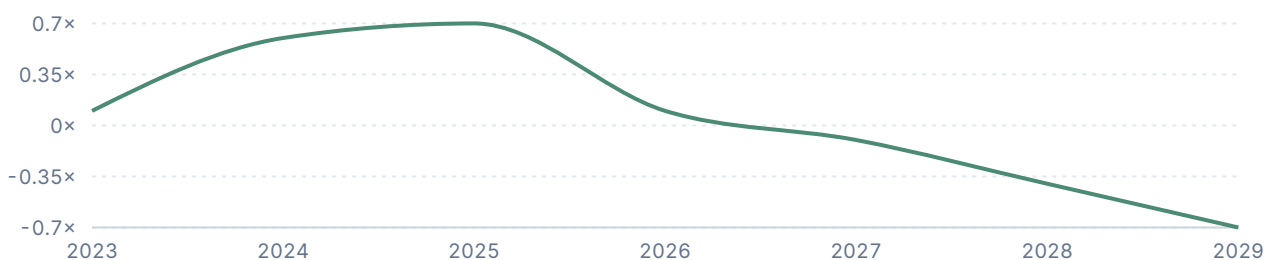
Free cash flow

DKK millions



Net debt / EBITDA

Ratio



KEY FINANCIALS

	2025	2026	2027	2028	2029
Net Sales	309,064	298,825	304,537	322,491	335,882
EBITDA	145,120	148,102	144,988	154,234	137,450
Comparable EBIT	127,658	122,444	122,869	131,579	113,580
Net Earnings	102,434	109,233	100,843	112,395	103,644
EPS (DKK)	23.1	24.6	22.7	25.3	23.3
DPS (DKK)	11.7	11.8	10.9	12.1	11.2

VALUATION

Target price derivation

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The rationale for the chosen valuation method and multiple, with the company's historical multiples.

SOTP is primary because marketed obesity, mature diabetes, rare disease and pre-revenue options have different growth, risk and capital economics. Marketed pools use 2026 EBITDA or EBIT multiples inside peer and historical brackets; optionality uses probability-weighted future scale economics discounted at 7%. The base target gives 100% weight to the SOTP, while group EV/EBITDA sensitivity shows the effect of disputing either EBITDA or the aggregate multiple.

What the market is pricing.

The current DKK 1,353.807bn EV is DKK 482bn below the target SOTP EV. Holding marketed-engine target values at DKK 1,301.5bn leaves only DKK 52.3bn for all options, versus about DKK 534bn in the base bridge. In this framework, the market prices roughly 10% of the report's probability-weighted option value, or assumes marketed EBITDA falls materially below the modelled DKK 137bn.

The market assumes Most pipeline economics are worth close to zero after cannibalisation. We assign a different reading: CKD and MASH already have positive outcomes or approvals, supporting DKK 195bn of discounted label-expansion value despite a 50% failure remainder. (NEJM, 24 May 2024; FDA/Novo, 28 January 2025; GOV.UK, 3 July 2026)

The market assumes Novo's prescription loss implies a permanent collapse in obesity patient scale. We assign a different reading: U.S. share is weak, but 4.1m patients and 90% oral-segment share support continued scale; the target uses 10.5x rather than a historical growth multiple. (Novo Q1 2026 report, 6 May 2026; Reuters, 14 August 2026)

Two enterprise values, two questions. The division pages allocate today's market enterprise value across the divisions; the target-price bridge states what each is worth on this report's own estimates. For Obesity Care — marketed franchise the two readings are 581 billion allocated and 277.6 billion warranted.

HISTORICAL AND FORECAST MULTIPLES

METRIC	2020A	2021A	2022A	2023A	2024A	2025A	2026E	NORM. AVG. 2020 - 2025 (OUTLIERS DROPPED)
P/E	23.55x	35.35x	38.40x	37.40x	27.53x	14.11x	12.21x	29.39x
P/E adj.	24x	35x	38x	37x	28x	14x	12x	29x
EV/EBITDA	16.59x	26.35x	26.02x	29.07x	20.95x	10.68x	9.14x	21.61x
EV/EBIT	18.35x	29.06x	28.58x	30.63x	22.34x	12.14x	11.06x	23.52x
EV/Sales	7.82x	12.10x	12.08x	13.53x	9.87x	5.01x	4.53x	10.07x
P/FCF	34.63x	77.96x	42.49x	45.35x	-291.31x	44.81x	10.25x	49.05x
P/BV	15.72x	23.87x	25.45x	29.37x	19.38x	7.45x	5.30x	20.21x
P/S	7.84x	11.99x	12.01x	13.47x	9.57x	4.68x	4.46x	9.93x
Dividend yield	4.0%	2.5%	2.4%	2.0%	1.6%	3.6%	3.9%	2.7%

VALUATION

Peers and target price bridge

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Validated peer multiples, each method's implied price and weight, and the sensitivity of the target.

PEER COMPARISON

COMPANY	2026E P/E	2026E EV/EBITDA	2026E EBIT MARGIN
Novo Nordisk	12.21x	9.14x	41.0%
Eli Lilly	66.1x	43.7x	32.8%
Sanofi	24.8x	8.1x	28.2%
AstraZeneca	24.4x	13.3x	34.0%
Roche	n/a	n/a	n/a
Amgen	24.6x	11.6x	43.7%
Zealand Pharma	18.8x	19.5x	26.9%
Viking Therapeutics	n/a	n/a	n/a
Sobi	25.0x	12.8x	30.4%
Peer median	24.6x	12.8x	31.6%

TARGET PRICE BRIDGE

METHOD	FORECAST METRIC	METRIC VALUE	SELECTED MULTIPLE	IMPLIED PRICE (12M DISC.)	WEIGHT
· Obesity Care — marketed franchise	2026E Modelled EBITDA	75,000	10.5x	177.3 DKK	
· Diabetes Care — marketed franchise	2026E Modelled EBITDA	62,000	8.0x	111.6 DKK	
· Rare Disease — marketed franchise	2026E Modelled EBIT	1,500	12.0x	4.1 DKK	
· Next-Generation Obesity and Metabolic Pipeline (probability-weighted)	Expected enterprise value	n/a	n/a	62.5 DKK	
· · moderate success	2032E Modelled EBITDA ...	60,000	12.0x	162.1 → 31.2 DKK	30% prob.
· · breakthrough	2032E Modelled EBITDA ...	120,000	12.0x	324.1 → 31.2 DKK	15% prob.
· Incretin Label-Expansion Optionality (probability-weighted)	Expected enterprise value	n/a	n/a	42.3 DKK	
· · moderate adoption	2032E Modelled EBITDA ...	45,000	10.0x	101.3 → 22.8 DKK	35% prob.
· · broad outcomes success	2032E Modelled EBITDA ...	90,000	10.0x	202.6 → 19.5 DKK	15% prob.
· Rare-Disease Pipeline Optionality (probability-weighted)	Expected enterprise value	n/a	n/a	11.4 DKK	
· · moderate success	2031E Modelled EBIT at s...	8,000	12.0x	21.6 → 6.8 DKK	45% prob.
· · high success	2031E Modelled EBIT at s...	16,000	12.0x	43.2 → 4.6 DKK	15% prob.
Sum of the parts	2026E present value Pro...	n/a	n/a	404.5 DKK	100%
Weighted target price				404.5 DKK	100%

VALUATION MAP

ROE vs P/BV — pricing versus profitability

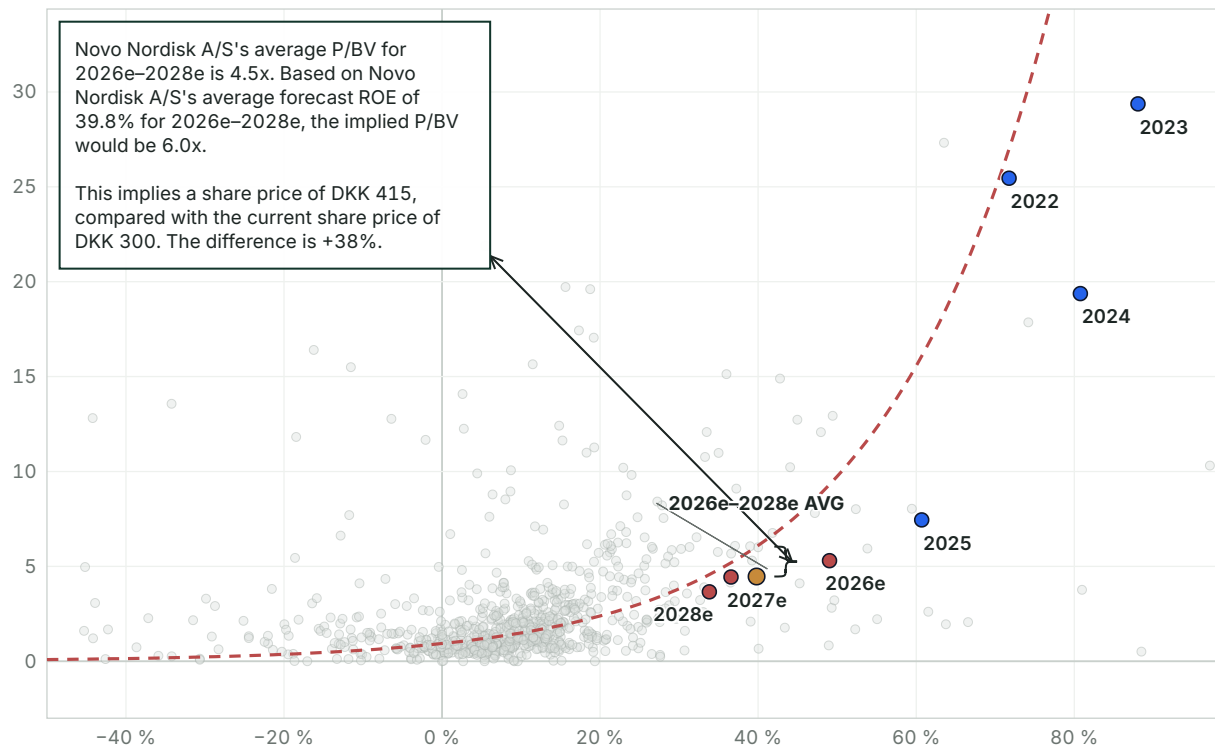
18

The whole Wisdom universe as context: how the market prices profitability on average, and what the company's forecast position would imply for the share price.

ROE 2025 vs P/BV 2025

Novo Nordisk A/S in the Wisdom universe (4 632 companies)

○ Universe ● History ● Forecast ● Forecast average



Novo's 2026–2028 average forecast ROE of 39.8% and average P/BV of 4.5x sit below the 6x universe curve implied by that return. Applying 6x to DKK 68.7 average forecast book value per share gives DKK 415.4, 38% above the rounded current price. The scatter supports the SOTP target, but buybacks, acquired intangibles and capital-cycle timing distort book value. It is therefore a cross-check rather than the primary method.

MULTIPLES & SENSITIVITY

19

Forward multiples and EBITDA × multiple grid

How the implied valuation responds to changes in EBITDA and the applied multiple.

FORWARD MULTIPLES	
	2026
P/E	12.2×
EV/EBITDA	9.1×
EV/EBIT	11.1×
P / Valuatum FOCF	10.2×
P/BV	5.3×
Dividend Yield	3.9%
Net Debt / EBITDA	0.1×

EBITDA × EV/EBITDA sensitivity						
2026E GROUP EBITDA (DKKM)						
Implied EV/EBITDA	10.4×	11.4×	12.4×	13.4×	14.4×	
→						
-20% DKK 118,482m	DKK 1,232,213m DKK 272.80 / sh	DKK 1,350,695m DKK 299.40 / sh	DKK 1,469,177m DKK 326.10 / sh	DKK 1,587,659m DKK 352.80 / sh	DKK 1,706,141m DKK 379.50 / sh	
-10% DKK 133,292m	DKK 1,386,237m DKK 307.40 / sh	DKK 1,519,529m DKK 337.50 / sh	DKK 1,652,821m DKK 367.50 / sh	DKK 1,786,113m DKK 397.50 / sh	DKK 1,919,405m DKK 427.50 / sh	
Base DKK 148,102m	DKK 1,540,261m DKK 342.10 / sh	DKK 1,688,363m DKK 375.50 / sh	DKK 1,836,465m DKK 408.80 / sh	DKK 1,984,567m DKK 442.10 / sh	DKK 2,132,669m DKK 475.50 / sh	
+10% DKK 162,912m	DKK 1,694,285m DKK 376.80 / sh	DKK 1,857,197m DKK 413.50 / sh	DKK 2,020,109m DKK 450.10 / sh	DKK 2,183,021m DKK 486.80 / sh	DKK 2,345,933m DKK 523.50 / sh	
+20% DKK 177,722m	DKK 1,848,309m DKK 411.50 / sh	DKK 2,026,031m DKK 451.50 / sh	DKK 2,203,753m DKK 491.50 / sh	DKK 2,381,475m DKK 531.50 / sh	DKK 2,559,197m DKK 571.50 / sh	

SCENARIO VALUATION

Bear / Base / Bull bridge

Revenue, EBITDA, multiple, EV, equity, value-per-share and upside in each scenario.

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SCENARIO BRIDGE								
	REVENUE	EBITDA	MARGIN	MULTIPLE	EV	EQUITY	VALUE / SHARE	UPSIDE
Bear	270,000	108,000	40.0%	12.1×	1,301,500	1,281,103	DKK 288.38	-3.9%
Base	298,825	148,102	49.6%	12.3×	1,817,554	1,797,157	DKK 404.54	+34.8%
Bull	360,000	190,000	52.8%	15.5×	2,939,795	2,919,398	DKK 657.15	+118.9%

KEY CONDITIONS

Bear requires simultaneous deterioration in volume share, net price and persistence, with no pipeline value. Base requires broadly stable marketed EBITDA, manufacturing execution and only partial pipeline success. Bull requires reimbursed patient expansion, better oral economics, positive pivotal differentiation and cash accumulation consistent with the followed model's shift to net cash from 2027. Each scenario follows the target-price bridge's own logic: profit engines as modelled, with Next-Generation Obesity and Metabolic Pipeline, Incretin Label-Expansion Optionality and Rare-Disease Pipeline Optionality valued at zero in Bear, at the bridge's probability weighting in Base, and at the best single case with certainty in Bull. The multiple column is therefore implied group EV/EBITDA, not a selected multiple. The code-verified 12-month target of DKK 404.5 sits between Bear and Base: Bear DKK 288.4, Base DKK 404.5, Bull DKK 657.2. Its method weights are shown in the Target Price Bridge.

THESIS BREAKER

A combined fall in obesity share below one-third and group EBIT margin below 35% before 2029 would invalidate the base scenario.

CATALYSTS

Monitoring checklist

21

Near-, medium- and long-term events that could change the recommendation.

CATALYST REGISTER				
	TIMING	SEGMENT	INDICATOR	VALUATION IMPACT
CagriSema FDA action and final label	NEAR-TERM	Next-Generation Obesity and Metabolic Pipeline	Approval, label breadth and titration	Resets 30% moderate-success probability
Oral Wegovy prescription and persistence trend	NEAR-TERM	Obesity Care – marketed franchise	Scripts, refill rate and net price	Tests volume-for-price trade-off
Awicli U.S. formulary adoption	NEAR-TERM	Diabetes Care – marketed franchise	Preferred access and weekly starts	Supports insulin migration value
Mim8 final regulatory status	NEAR-TERM	Rare-Disease Pipeline Optionality	FDA/EMA outcome and label	Clarifies advanced pipeline value
2027 U.S. semaglutide price reset	MEDIUM-TERM	Obesity Care – marketed franchise	Net revenue and covered lives	Determines price-volume elasticity
Etavopivat and zaltenibart filings	MEDIUM-TERM	Rare-Disease Pipeline Optionality	On-time submissions and datasets	Raises or lowers 60% success pool
Confirmatory MASH and reimbursement progress	MEDIUM-TERM	Incretin Label-Expansion Optionality	Hard outcomes and funded access	Tests incremental label economics
Kalundborg, Athlone and Clayton ramp	LONG-TERM	Obesity Care – marketed franchise	Saleable doses and utilisation	Converts capex into margin and supply
READ - THROUGH Medium-term catalysts will test whether Novo can exchange lower prices for larger persistent patient populations and whether label expansion creates incremental revenue rather than relabelling existing patients. Manufacturing milestones matter only when qualified capacity produces saleable doses at acceptable cost; construction completion alone is not a value event.				

RISKS

Risk register

22

Each risk's business division, mechanism, early-warning trigger, impact band and structural type.

RISK REGISTER					
	SEGMENT	MECHANISM	EARLY WARNING	IMPACT	TYPE
Lilly extends its U.S. efficacy and share lead	Diabetes Care – marketed franchise	Lower starts, weaker formulary access and price concessions	Novo U.S. share below one-third	HIGH	STRUCTURAL
Price cuts outrun covered-patient growth	Diabetes Care – marketed franchise	Net revenue falls despite rising prescription volume	Revenue per patient-year declines >15%	HIGH	THESIS BREAKER
Long-term persistence remains near 8%	Incretin Label-Expansion Optionality	High churn lowers lifetime value and raises reacquisition cost	Two-year refill cohorts fail to improve	HIGH	STRUCTURAL
Manufacturing quality delays capacity	Diabetes Care – marketed franchise	Qualified-dose shortages and underabsorbed capital	Further FDA action at fill-finish sites	HIGH	MANAGEABLE
CagriSema and amycretin lack differentiation	Next-Generation Obesity and Metabolic Pipeline	Approval without premium share or complete clinical failure	Inferior efficacy, tolerability or titration	HIGH	STRUCTURAL
Outcome labels fail to secure reimbursement	Incretin Label-Expansion Optionality	Approved indications add little incremental patient access	Restrictive criteria and low funded uptake	MEDIUM	MANAGEABLE
Rare launches cannot offset legacy erosion	Rare-Disease Pipeline Optionality	Low utilisation prevents margin recovery	Rare Disease EBIT margin stays below 8%	MEDIUM	MANAGEABLE
Semaglutide protection erodes early	Obesity Care – marketed franchise	Generic or compounded alternatives accelerate price pressure	Adverse patent outcomes or broad substitution	HIGH	THESIS BREAKER
READ-THROUGH Manufacturing and rare-disease execution risks are more manageable because regulatory status, supply and margins can be monitored. Pipeline risk is deliberately limited through large zero-value failure outcomes, although positive pipeline values would still decline rapidly after adverse pivotal or regulatory events.					

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APPENDIX

Income statement

INCOME STATEMENT — DKK MILLIONS						
	2023	2024	2025	2026 E	2027 E	2028 E
Net Sales	232,261	290,403	309,064	298,825	304,537	322,491
EBITDA	108,091	136,884	145,120	148,102	144,988	154,234
EBITDA margin	46.5%	47.1%	47.0%	49.6%	47.6%	47.8%
Depreciation	-5,517	-8,545	-17,462	-25,658	-22,119	-22,655
Operating Profit (EBIT)	102,574	128,339	127,658	122,444	122,869	131,579
EBIT margin	44.2%	44.2%	41.3%	41.0%	40.3%	40.8%
Net financial items	2,100	-1,148	2,882	6,761	1,476	7,010
Pre-tax Profit	104,674	127,191	130,540	129,205	124,345	138,589
Net Earnings	83,683	100,988	102,434	109,233	100,843	112,395
PER SHARE						
EPS	18.7	22.7	23.1	24.6	22.7	25.3
DPS	14.2	9.9	11.7	11.8	10.9	12.1
Payout ratio	75.8%	43.7%	50.5%	47.8%	47.8%	47.8%

E = Valuatum estimate (not yet actualised)

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APPENDIX

Balance sheet

BALANCE SHEET — DKK MILLIONS						
	2023	2024	2025	2026 E	2027 E	2028 E
ASSETS						
Tangible assets	90,961	162,488	208,378	189,479	193,100	204,485
Intangibles	55,942	91,026	130,053	125,744	128,148	135,703
Goodwill	4,464	20,064	—	—	—	—
Non-current assets	154,460	280,271	346,802	323,594	329,620	348,559
Inventories	31,811	40,849	49,623	45,898	46,775	49,533
Receivables	93,443	104,393	96,366	99,722	101,233	105,983
Cash & equivalents	14,392	15,655	26,464	22,270	22,696	24,034
Current assets	139,646	160,897	172,453	167,890	177,334	224,503
Total Assets	314,486	465,795	542,902	515,131	530,600	596,709
EQUITY & LIABILITIES						
Equity	106,561	143,486	194,047	251,519	300,166	364,375
Long-term debt	15,976	84,246	111,705	17,160	—	—
Short-term debt	5,304	11,775	12,017	17,160	0	0
Long-term liabilities	22,382	92,589	119,347	24,802	7,642	7,642
Current liabilities	169,655	217,528	215,661	223,852	207,128	208,498
Total liabilities & equity	314,486	465,795	542,902	515,131	530,600	596,709
CAPITAL STRUCTURE & RATIOS						
Net debt	12,614	87,132	104,494	20,397	-20,273	-59,404
Capital invested	113,449	223,852	291,305	263,569	270,841	295,388
Equity ratio	33.9%	30.8%	35.7%	48.8%	56.6%	61.1%
Gearing	11.8%	60.7%	53.8%	8.1%	-6.8%	-16.3%
Net debt / EBITDA	0.1×	0.6×	0.7×	0.1×	-0.1×	-0.4×
Current ratio	0.8	0.7	0.8	0.8	0.9	1.1

E = Valuatium estimate (not yet actualised)

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APPENDIX

Cash flow

CASH FLOW — DKK MILLIONS						
	2023	2024	2025	2026 E	2027 E	2028 E
OPERATIONS						
Cash from operations (model)	113,127	130,947	117,040	138,308	121,009	128,912
Operating cash flow (Valuatum calculation)	107,401	122,875	116,944	132,592	119,812	123,227
Change in working capital	-14,178	-21,414	2,856	-3,417	1,953	6,137
INVESTING & FREE CASH						
Gross capex	40,341	134,356	83,993	2,450	28,144	41,594
Capex (ex. M&A)	-40,341	-134,356	-83,993	-2,450	-28,144	-41,594
Cash after capex (CFO - gross capex)	72,786	-3,409	33,047	135,858	92,865	87,318
Free operating cash flow (Valuatum def.)	69,000	-9,544	32,250	130,142	91,668	81,633
Free cash flow to firm	69,000	-9,544	32,250	130,142	91,668	81,633
FINANCING						
CF from financing	-57,446	13,655	-24,403	-140,053	-85,809	-47,657
Dividends paid	-50,516	-63,432	-44,094	-51,761	-52,195	-48,186
Net change in cash	15,340	10,246	8,644	-4,194	7,055	39,661

E = Valuatum estimate (not yet actualised)

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APPENDIX

Quarterly snapshot

HISTORICAL QUARTERS — LAST TWO COMPLETED YEARS, DKK MILLIONS

	Q1/24	Q2/24	Q3/24	Q4/24	Q1/25	Q2/25	Q3/25	Q4/25
Net Sales	65,349	68,060	71,311	85,683	78,087	76,857	74,976	79,144
EBITDA	34,760	34,779	35,972	31,373	42,621	38,282	26,919	37,298
EBITDA margin	53.2%	51.1%	50.4%	36.6%	54.6%	49.8%	35.9%	47.1%
Comparable EBIT	31,846	25,934	33,822	36,737	38,791	33,449	23,682	31,736
EBIT margin	48.7%	38.1%	47.4%	42.9%	49.7%	43.5%	31.6%	40.1%
Net Earnings	25,407	20,050	27,301	28,230	29,034	26,503	20,006	26,891
EPS	5.7	4.5	6.1	6.3	6.5	6	4.5	6.1

CURRENT YEAR — QUARTERLY, DKK MILLIONS

	Q1/26	Q2/26	Q3/26 E	Q4/26 E
Net Sales	96,823	78,488	72,909	50,605
EBITDA	63,154	37,365	23,757	23,826
EBITDA margin	65.2%	47.6%	32.6%	47.1%
Comparable EBIT	59,618	27,061	19,012	16,753
EBIT margin	61.6%	34.5%	26.1%	33.1%
Net Earnings	48,557	20,989	24,006	15,572
EPS	10.9	4.7	5.4	3.5

E = Valuatum estimate (not yet actualised)

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SOURCES & DISCLAIMER

Sources, assumptions, and standard disclaimer

SOURCES REGISTER			
	VALUE	CATEGORY	USED IN
Financial statements and forecast model	Valuatum Wisdom model 10516, snapshot period 2026	FINANCIAL SNAPSHOT	Financials, forecasts & valuation
Share-price history and market profile	Financial Modeling Prep, NOVO-B.CO financialmodelingprep.com	MARKET DATA	Price chart & market facts
Financial report for the period 1 January 2025 to 31 Decemb...	attachment.news.eu.nasdaq.com/.../a1fe2e2363f592299000744f98161e5a5	OFFICIAL SOURCE	Reported segment table
Maziar Mike Doustdar succeeded Lars Fruergaard Jørgensen as...	www.novonordisk.com	OFFICIAL SOURCE	Novo Nordisk

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